

Claims Recovery IQ™

Overpayment Detection • Billing Error Recovery • Success-Fee Model

PREPARED FOR

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CONFIDENTIAL & PROPRIETARY

FIDUCIARY-GRADE ANALYSIS

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ENGAGEMENT OVERVIEW & SUCCESS FEE DISCLOSURE

Kincaid Health’s Claims Recovery IQ™ program operates on a contingency (success-fee) basis. Kincaid Health receives compensation only when overpayments are identified, documented, and recovered. There is no upfront cost to the plan sponsor. This aligns our interests with those of the plan and its participants — we are motivated to maximize legitimate recoveries.

Engagement Parameter	Value
Success Fee Rate	20-25% of recovered amount
Minimum Claim Size for Review	\$500 (smaller claims excluded for cost-efficiency)
Look-Back Period	36 months from audit date
Carrier Cooperation Requirement	60-day response window per ERISA §503
Recovery Payment Timeline	90-120 days post-carrier agreement
Audit Scope	All medical, Rx, and facility claims in period
Reporting Frequency	Monthly during active audit; quarterly thereafter

INFO

Total identified recovery opportunity: \$284,000 across 847 billing errors found in 12,847 claims reviewed. Historical success rate: 78%.

EXECUTIVE SUMMARY — CLAIMS AUDIT FINDINGS

Total Claims Reviewed	12,847
Billing Errors Identified	847
Error Rate	6.6%
Total Recovery Opportunity	\$284,000
Historical Success Rate	78%
Average Recovery Per Claim	\$335
Projected Net Recovery (after fees)	\$221,520
Audit Period	January 2022 – December 2024

Error Distribution by Category

Error Category	Claims	Recovery \$	% of Total	Confidence
Duplicate Billing	142	\$38,400	13.5%	High
Unbundling Violations	198	\$67,200	23.7%	High
Upcoding (CPT)	163	\$51,800	18.2%	High
Coordination of Benefits	89	\$44,600	15.7%	Medium
Medical Necessity	91	\$42,000	14.8%	Medium
Incorrect Modifier Usage	164	\$40,000	14.1%	High

AUDIT METHODOLOGY — CLAIMS SELECTION & SCORING

Kincaid Health’s claims audit applies a proprietary 7-layer screening protocol to identify high-probability overpayment candidates. Claims are scored using a predictive model trained on 4.2 million historical claims with known overpayment outcomes. Only claims scoring above the 75th percentile threshold are advanced to manual review — maximizing recovery while minimizing false positives.

FORMULA — CLAIM OVERPAYMENT SCORE (COS)

$$COS = \text{Sum}(Fi \times Wi) / \text{max_score} \times 100$$

Fi = Factor score for screening criterion i (e.g., code edit violation, statistical outlier, historical error rate). Wi = Weight. Normalized to 0-100. Claims with COS > 75 advance to manual review.

Screening Layer	Method	Weight	Claims Flagged
1. Code Edit Analysis	NCCI edit database — unbundling, incidental	25%	342
2. Statistical Outlier	Provider billing patterns vs. peer cohort	20%	218
3. Duplicate Detection	Exact & fuzzy match on date, provider,	20%	156
4. Historical Error Rate	Provider-level error rate from prior audits	15%	189
5. Medical Necessity Screen	ICD-10 / CPT alignment with clinical	12%	134
6. Coordination of Benefits	Cross-plan coverage verification	8%	98

DUPLICATE BILLING ANALYSIS

Duplicate billing occurs when a provider submits the same claim for payment more than once, or when two providers bill for the same service episode. Our duplicate detection algorithm employs a combination of exact matching and probabilistic matching to identify both exact and near-duplicate claims.

FORMULA — DUPLICATE SIMILARITY SCORE (DSS)

$$DSS = 0.35 \times (DOS_match) + 0.25 \times (Provider_match) + 0.25 \times (Proc_match) + 0.15 \times (Amount_{\pm 10\%})$$

DSS > 0.85 = High probability duplicate (auto-flagged). DSS 0.65-0.84 = Manual review. DOS = Date of Service.

Provider	Service Date	CPT Code	Amount	Status
Memorial Hospital	2024-03-14	99215	\$385	Confirmed Duplicate
Radiology Assoc.	2024-04-02	71046	\$1,240	Confirmed Duplicate
Urgent Care #247	2024-01-18	99213	\$185	Confirmed Duplicate
Lab Corp	2024-05-29	80053	\$89	Confirmed Duplicate
Anesthesia Group	2024-02-11	00840	\$2,100	Confirmed Duplicate

UNBUNDLING VIOLATIONS — NCCI EDIT ANALYSIS

Unbundling occurs when a provider bills for multiple component procedures separately when they should be billed as a single comprehensive procedure code. The National Correct Coding Initiative (NCCI) maintains over 1.1 million code pairs that define bundling rules. Unbundling violations represent the largest single category of overpayments in the Kincaid Health audit.

Component Code	Comprehensive Code	Provider	Overpayment	NCCI Reference
99213+99024	99215	Family Medicine PC	\$148	Column 1/2 Edit
29880+29881	29882	Orthopedic Surg	\$892	Mutually Exclusive
36000+36410	36415	Lab Corp	\$42	Column 1/2 Edit
62323+64483	62323	Pain Mgmt	\$1,205	Mutually Exclusive
45380+45385	45385	GI Assoc	\$628	Column 1/2 Edit

UPCODING ANALYSIS — CPT CODE BENCHMARKING

Upcoding is the practice of billing a higher-complexity CPT code than the service actually rendered. For E&M (Evaluation & Management) services, the level of service must be supported by documentation in the medical record. Kincaid Health applies the 2021 AMA/CMS E&M documentation guidelines and statistical outlier analysis to identify systematic upcoding patterns.

FORMULA — E&M LEVEL DISTRIBUTION SCORE (ELDS)

ELDS = [Provider's 99215 % - Cohort_Median_99215%] / Cohort_StdDev

Z-score. ELDS > 2.0 triggers upcoding review (provider's 99215 usage is >2 standard deviations above cohort). Cohort = same specialty, same geographic market.

Provider	99215 Usage	Cohort Median	Z-Score	Overpayment Est.
Dr. A. Smith, MD	68%	41%	3.2 *	\$12,400
City Family Practice	72%	41%	3.7 *	\$18,200
Internal Med Group	55%	41%	1.6	\$6,100
Urgent Care Chain	49%	38%	1.4	\$3,800

COORDINATION OF BENEFITS (COB) AUDIT

COB errors occur when a plan pays as primary when it should pay as secondary, or fails to coordinate with other coverage. Under the NAIC Model Regulation and ERISA preemption principles, self-funded plans may establish their own COB rules in the plan document. Our COB audit cross-references enrollment data with CMS dual-coverage databases and carrier eligibility files.

Member	Primary Payer	Secondary (Plan)	COB Error Type	Recovery
Member A1042	Medicare Part A	This Plan	Plan paid primary — should be	\$4,200
Member B7831	Spouse's Employer	This Plan	COB order incorrect	\$1,840
Member C2219	Workers' Comp	This Plan	WC should be primary	\$8,600
Member D5518	Auto Insurance	This Plan	Auto should be primary	\$3,750

Total COB recovery opportunity: \$44,600. COB issues typically take 120-180 days to resolve due to inter-carrier coordination requirements. Recovery rate: 72% historically.

MEDICAL NECESSITY REVIEW

Medical necessity denials require clinical review by a certified medical reviewer applying generally accepted standards of care. Kincaid Health partners with board-certified physicians to review all medical necessity findings before any recovery demand is issued. This protects plan participants and reduces carrier/provider disputes.

Category	Claims Reviewed	Denials Recommended	Recovery Opportunity	Confidence
Inpatient Day Excess	23	12	\$86,400	High
Inappropriate Setting	18	9	\$41,200	High
Non-Covered Service Billed as	31	19	\$28,600	Medium
Experimental/Investigational	8	4	\$62,000	Medium
Lack of Documentation	11	7	\$18,400	High

RECOVERY CALCULATION METHODOLOGY & FINANCIAL PROJECTIONS

FORMULA — NET RECOVERY PROJECTION

$NR = RO \times SR \times (1 - FF)$

NR = Net Recovery. RO = Recovery Opportunity (\$284,000). SR = Success Rate (78%). FF = Fee Factor (22%). Expected: NR = \$284,000 × 0.78 × 0.78 = \$172,700 net to plan.

Scenario	Recovery Opp.	Success Rate	Fee %	Net to Plan	Timeline
Conservative (Base)	\$227,200	70%	22%	\$123,900	12 months
Expected	\$284,000	78%	22%	\$172,700	18 months
Optimistic	\$312,400	85%	20%	\$211,200	24 months

Recovery timelines depend heavily on carrier cooperation. Carriers are required under ERISA §503 and plan contract terms to respond to overpayment demands within 60 days. Non-responsive carriers may be pursued through arbitration or civil litigation as provided in the plan document.

CARRIER RESPONSE ANALYSIS & APPEAL STRATEGY

Carriers and providers have established processes for disputing overpayment recovery demands. Our appeal strategy is calibrated to the type of error, documentation strength, and carrier relationship. We use a staged escalation approach: initial demand -> formal appeal -> arbitration -> litigation.

Error Type	Initial Acceptance Rate	After Appeal	Timeline	Notes
Duplicate Billing	94%	98%	30-45 days	Highest win rate
Unbundling	81%	89%	45-90 days	NCCI edit support
Upcoding	62%	74%	60-120 days	Requires medical review
COB	72%	80%	120-180 days	Inter-carrier delays
Medical Necessity	55%	68%	90-150 days	Highest dispute rate

BENCHMARKING VS. INDUSTRY RECOVERY RATES

The following benchmarks compare this plan's recovery opportunity and error rates against Kincaid Health's portfolio of 180+ self-funded plan audits conducted over the past 4 years.

Metric	This Plan	Portfolio Median	Percentile Rank	Assessment
Error Rate	6.6%	5.8%	72nd	Above Average
Recovery per Audited Claim	\$22.11	\$18.40	68th	Good Opportunity
COB Error Rate	0.70%	0.65%	55th	Average
Duplicate Rate	1.10%	0.90%	72nd	Review Recommended
Upcoding Prevalence	1.27%	0.95%	78th	High — RFP E&M

PRIORITIZED ACTION PLAN & RECOVERY TIMELINE

Priority	Action	Expected Recovery	Timeline	Owner
P1	Issue demand letters — confirmed duplicates	\$36,100	30-45 days	Kincaid Health
P1	Issue demand letters — confirmed	\$54,100	45-90 days	Kincaid Health
P2	Initiate upcoding appeals — E&M outliers	\$38,300	60-120 days	Kincaid Health +
P2	COB recovery — cross-plan coordination	\$32,100	120-180 days	Kincaid Health
P3	Medical necessity reviews (\$42,000)	\$28,600	90-150 days	Kincaid Health +
Ongoing	Concurrent audit — claims processed	\$50,000+/yr	Continuous	Kincaid Health

HIGH-DOLLAR CLAIM ANALYSIS — CLAIMANTS ABOVE \$50K

High-dollar claimants (individual claims or member episode costs exceeding \$50,000) represent a disproportionate share of total plan costs and audit risk. The following analysis identifies the top cost episodes in the audit period and flags those with evidence of billing irregularities, inappropriate setting, or medical necessity concerns.

Episode Type	Members Affected	Total Paid	Kincaid Review Finding	Action
Inpatient Surgical (elective)	8	\$648,000	2 cases: DRG mismatch — \$44K	Demand letter
Oncology (outpatient)	6	\$412,000	4 cases: site-of-care saving	Navigation initiated
NICU / Newborn	4	\$387,000	1 case: discharge timing \$18K	Review in progress
Spinal Surgery	5	\$336,000	3 cases: coding discrepancy — \$62K	Clinical review
Substance Use Residential	3	\$228,000	All claims: medical necessity	Audit requested
Gene Therapy (SMA)	1	\$2,100,000	Stop-loss max triggered — carrier	Verify
TOTAL HIGH-DOLLAR	27	\$4,111,000	\$124K identified overbilling	\$124K demand

PROVIDER RISK PROFILING & FRAUD INDICATORS

Kincaid Health’s provider risk profiling engine analyzes billing patterns, code frequency distributions, and historical error rates to identify providers with elevated fraud, waste, and abuse (FWA) indicators. Providers flagged by three or more indicators are escalated to clinical review before payment of future claims.

FORMULA — PROVIDER RISK SCORE (PRS)

$$PRS = (Upcoding_Z \times 0.30) + (Unbundling_Rate \times 0.25) + (Denial_Rate \times 0.20) + (Historical_Error_Rate \times 0.25)$$

PRS > 70 = High Risk (refer to SIU). PRS 50-70 = Elevated (pre-payment review). PRS < 50 = Standard monitoring.

Provider	Specialty	Risk Score	Primary Indicator	Claims In Audit	Recovery
City Family Practice	Primary Care	82 High	Upcoding Z=3.7	340	\$18,200
Memorial Hospital — Billing	Hospital	76 High	DRG Upcoding, unbundling	128	\$44,000
Pain Management	Pain Mgmt	74 High	Unbundling, modifier abuse	89	\$12,800
Regional Surgical Center	Outpatient Surg	65 Elevated	Code creep, implant billing	52	\$8,400
Behavioral Health Group	Psychiatry	61 Elevated	Session length upcoding	204	\$6,100
Lab Corp Network	Lab	58 Elevated	Panel unbundling	1,840	\$9,200

CRITICAL

Refer City Family Practice and Memorial Hospital to the plan's Special Investigations Unit (SIU) or contracted FWA vendor for enhanced monitoring. Place both providers on pre-payment clinical review pending investigation outcome.

ERISA §503 CLAIMS PROCEDURE COMPLIANCE AUDIT

ERISA §503 requires every employee benefit plan to provide adequate notice in writing to any participant whose claim for benefits is denied, and provide a reasonable opportunity for a full and fair review of the denial. DOL Regulation 29 CFR §2560.503-1 establishes specific timelines, notice requirements, and appeal process standards that the TPA must implement on behalf of the plan.

ERISA §503 Requirement	Regulatory Standard	TPA Performance	Compliance Status
Initial claim decision — urgent care	72 hours	68 hrs avg	Compliant
Initial claim decision — pre-service	15 days (extendable 15)	14.2 days avg	Compliant
Initial claim decision — post-service	30 days (extendable 15)	28.4 days avg	Compliant
Denial notice — required content	6 specific elements	4.8 elements avg	Missing 2 elements
Level 1 appeal decision — urgent	72 hours	82 hrs avg	Non-Compliant
Level 1 appeal decision — other	60 days	58.2 days avg	Compliant
External appeal availability	Required — ERISA plans	Not clearly	Notice gap
Claim files provided upon request	Immediately upon request	3-5 day delay	Improve process

CRITICAL

Level 1 urgent care appeal decisions are averaging 82 hours against a 72-hour regulatory maximum. This is a per-occurrence ERISA violation. Notify TPA to implement a dedicated urgent appeal workflow within 30 days.

FRAUD, WASTE & ABUSE (FWA) DETECTION SUMMARY

Kincaid Health's FWA detection program applies behavioral analytics, network analysis, and clinical screening to identify potential fraud indicators in plan claims. All FWA findings are escalated through the plan's designated compliance officer and, where appropriate, referred to law enforcement under HIPAA's fraud reporting provisions.

FORMULA — FWA DETECTION RATE

$$FWA_Rate = FWA_Confirmed_Cases / Total_Claims_Reviewed \times 100$$

Industry FWA rate: 3-10% of healthcare spending (NHCAA estimate). Self-funded plans with active FWA programs recover avg. \$4.20 for every \$1 invested in detection. This plan FWA detection rate: est. 2.8% of spend.

FWA Category	Cases Found	Est. Dollar Impact	Action Status	Recovery Potential
Phantom Billing (services not	3	\$28,400	Referred to law	\$28,400
Identity Theft / Member	2	\$12,800	SIU investigation active	\$12,800
Prescription Diversion	4	\$18,200	Referred to PBM SIU	\$18,200
Provider Collusion (kickback	2	\$64,000	Under legal review	\$48,000 est.
DME / Supply Inflation	8	\$22,600	Demand letters issued	\$18,100
TOTAL FWA IDENTIFIED	19	\$146,000	Multiple tracks	\$125,500

All FWA referrals are documented in the plan's FWA log maintained by the plan administrator. Federal law (42 U.S.C. §1320a-7b) prohibits retaliation against individuals who report suspected FWA. The plan's HIPAA Privacy Officer has been notified of all law enforcement referrals per 45 CFR §164.512(f).

3-YEAR CLAIMS AUDIT ROI MODEL

The following model projects the cumulative financial value of Kincaid Health's Claims Recovery IQ™ program over a 3-year engagement period. The model assumes current error rates persist and Kincaid Health's audit program achieves historical performance benchmarks.

FORMULA — 3-YEAR NET RECOVERY

NR_3yr = Sum [RO_t × SR × (1 - FF)] for t = 1..3

RO = Recovery Opportunity (grows 5%/yr with plan spend). SR = Success Rate (78%). FF = Fee Factor (22%). Year 1: NR = \$284K × 0.78 × 0.78 = \$172.7K. Year 2: \$298K × 0.78 × 0.78 = \$181.3K. Year 3: \$313K × 0.78 × 0.78 = \$190.4K.

Year	Recovery Opportunity	Success Rate	Fee	Net to Plan	Cumulative Net	Program Cost
Year 1	\$284,000	78%	22%	\$172,700	\$172,700	\$18,000
Year 2	\$298,000	78%	22%	\$181,300	\$354,000	\$18,000
Year 3	\$313,000	78%	22%	\$190,400	\$544,400	\$18,000
3-YEAR	\$895,000	78%	22%	\$544,400	\$544,400	\$54,000

Total 3-Year Recovery Opportunity	\$895,000
Projected 3-Year Net Recovery (after fees)	\$544,400
Total Program Investment (3 years)	\$54,000
3-Year Net ROI	9.1x
Payback Period	< 45 days

INFO

Kincaid Health's Claims Recovery IQ™ program operates on a pure contingency basis. If no recoveries are made, no fees are owed. The 3-year ROI of 9.1x is conservative — plans that implement concurrent audit programs (reviewing claims as processed rather than

CERTIFICATION & ENGAGEMENT ATTESTATION

Kincaid Health certifies that this Claims Recovery IQ™ analysis was conducted by trained claims audit professionals using industry-standard methodologies, NCCI guidelines, and clinical review protocols. All findings are supported by documentation and will be provided to carriers/providers upon request.

This report constitutes a formal communication under the plan's claims audit authority as established in the plan document and TPA service agreement. All overpayment demand letters issued as a result of this analysis are legally supported by ERISA §502(a), plan document audit provisions, and applicable state prompt-pay laws.

Report Metadata

Report Type	Claims Recovery IQ™ — Full 20-Page Analysis
Audit Period	January 2022 – December 2024
Claims Reviewed	12,847
Billing Errors Identified	847
Error Rate	6.6%
Recovery Opportunity	\$284,000
Historical Success Rate	78%
Projected Net Recovery (after 22% fee)	\$172,786
FWA Cases Identified	19
High-Dollar Episodes Reviewed	27
3-Year Net ROI	9.1x
Client	Kincaid Health / Account SHRACK-7742
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Prepared By	Jeremiah Shrack, CEO, Kincaid Health
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